HeartSafe Agent – Execution Plan (Nov 5–13)

Nov 5, 2026 · @yassmin

Positioning (after mentor meeting, Nov 7)

HeartSafe is an equity-aware trust and coordination layer on top of an existing remote-monitoring platform, not another AI cardiac monitor. Our anchor is Medly (UHN, Toronto): it already collects daily measurements and alerts the care team, and HeartSafe makes those alerts safer and accountable.

1. Medly collects daily weight, blood pressure, heart rate and symptoms, and raises its own normal / caution / critical alerts.
2. HeartSafe adds ECG and imaging signals, checks how reliable every input and model output is, and prepares an explainable review.
3. The clinical team reviews, acknowledges and decides. HeartSafe never makes the clinical decision.
4. HeartSafe tracks every escalation to resolution, documents it, and monitors equity across communities.

Who it is for: adults with heart failure enrolled in remote monitoring, including remote and Indigenous communities. The devices and the agent complement scheduled follow-up visits; they do not replace them.

Medly is not open source (no public code or API found), so we build a Medly-compatible adapter and run it on a simulated Medly-style feed. A real deployment would connect to Medly's data in partnership with UHN.

The mentor sees the reliability and equity monitoring layer as our strongest differentiator. Open items: Oumnia's connected-therapeutics link; follow-ups go to her on WhatsApp.

Goal and MVP scope

By Nov 13 we demo HeartSafe running on a Medly-style feed for a large simulated cohort, with every escalation tracked, an equity audit, and results against Medly alone.

Must work (MVP)

- Medly adapter: reads daily vitals, symptoms and Medly's own alert level. A simulated cohort (about 2,000 patients × 90 days) follows Medly's published inputs.

- Reliability layer: data-quality checks on Medly readings (missing days, implausible values, device changes), plus ECG and cardiac MRI model reliability (quality, uncertainty, shift, plausibility), leading to accept / repeat measurement / escalate.
- Explainable review: for each escalation, a structured review (why it was flagged, the trend, reliability caveats, suggested next step). An LLM writes the wording, with a template fallback.
- Follow-up tracker: Created → Acknowledged → Reviewed → Resolved, with deadlines, reminders and an audit log. Only a clinician closes a case.
- Equity monitor: alert, unreliable-data, missed-escalation and time-to-acknowledge rates by remoteness, sex, age band and device, with bootstrap 95% CIs. Safeguards need administrator approval.
- Evaluation against a Medly-alone baseline.
- Dashboard: clinician queue, case review, case timeline, equity audit.

Large-scale data: PTB-XL (21,799 ECGs) for ECG, M&Ms for cardiac MRI, and the simulated Medly-style cohort for daily vitals.

Stretch: connected-therapeutics ideas (missed readings as an adherence signal), real notifications.

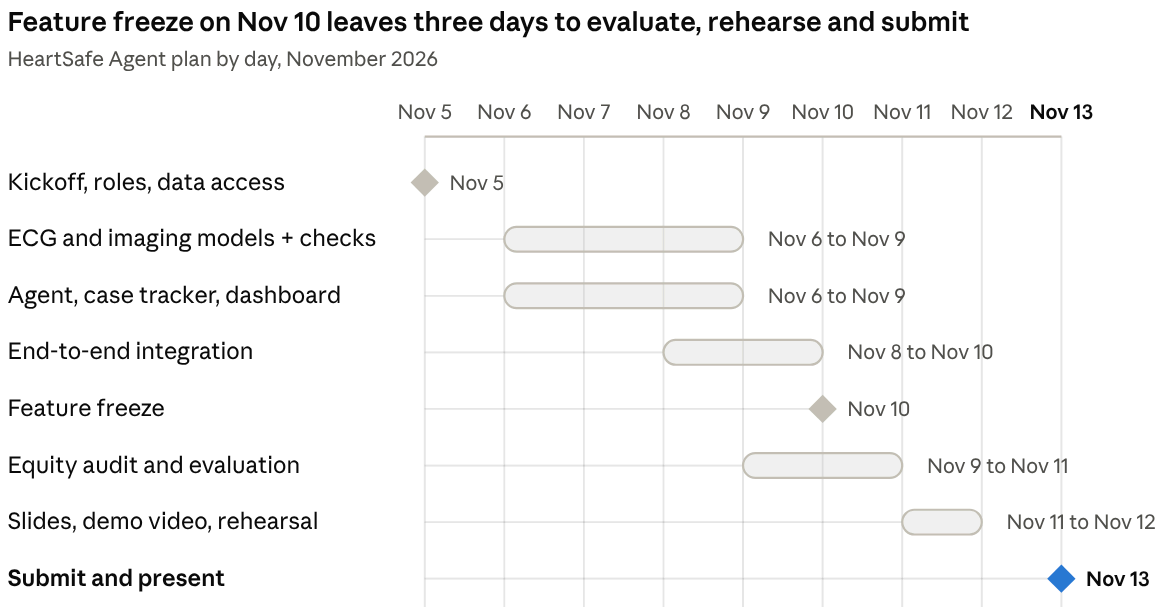
Team roles

Each person owns one workstream end to end and one slice of the presentation. The split below is a proposal to confirm at kickoff.

Person	Owns	Also owns
Yassmin Abdelaziz	Imaging path (nnU-Net, M&Ms, volumes/EF, imaging reliability) and equity audit	Integration lead; results slides
Ana Karen Luque Flores	ECG path (PTB-XL training, PhysioNet 2017 shift test, ECG reliability)	Evaluation harness and baseline
Vertika Chaudhary	Agent orchestrator, case tracker, dashboard	Demo script and backup video
Mentor	Scope check, clinical plausibility of escalation rules, judging expectations	Final rehearsal feedback

Everyone codes against one shared interface: each model returns `{prediction, probability, uncertainty, quality, shift_score, metadata}`. That lets the agent be built on mock outputs from day one.

Timeline



execution timeline · Nov 5–13

Models and the agent are built in parallel on mock outputs (Nov 6–9) so integration can start Nov 8. Nothing new is added after the Nov 10 freeze; the last three days are evaluation, slides and rehearsal.

Day	Yassmin	Ana Karen	Vertika
Thu Nov 5	Kickoff call; request M&Ms access; set up shared repo	Download PTB-XL and PhysioNet 2017	Agent skeleton on mock outputs
Fri Nov 6	Start nnU-Net training on the cluster	Train ECG baseline	Policy engine + case tracker
Sat Nov 7	Imaging plausibility + shift checks	ECG quality, uncertainty, shift test	LLM tools + review drafts
Sun Nov 8	Integration: swap mocks for real outputs	Integration	Dashboard views

Day	Yassmin	Ana Karen	Vertika
Mon Nov 9	Volumes/EF, uncertainty maps; subgroup metrics	Simulated cohort + fault injection	Reminders, audit log
Tue Nov 10	Safeguard loop; **feature freeze**	Baseline vs agent runs	Bug fixes; mentor demo review
Wed Nov 11	Results figures	Evaluation table	Demo script; record backup video
Thu Nov 12	Slides final	Slides final	Mentor rehearsal; fixes only
Fri Nov 13	Submit and present	Submit and present	Submit and present

Workstream tasks

Tick these off as they land; each has a concrete done-check.

Imaging (Yassmin)

- [x] ~~Download M&Ms, keep vendor and centre metadata; pick a subset that fits the GPU budget~~
- [x] ~~Train nnU-Net (2d config, one fold, reduced epochs) on the cluster; done = Dice reported per vendor~~
- [x] ~~Compute LV/RV volumes and EF; softmax-entropy uncertainty map per case~~
- [x] ~~Plausibility rules (EF 0–100, volume ranges, one connected LV component) and vendor-shift score~~
- [x] ~~Export cached outputs for every test case in the shared interface format~~

Imaging results (Nov 7, SDRE H200; nnU-Net 2d, fold 0, 250 epochs in 63 min):

Vendor (held out)	Seen in training	n	Dice LV	Dice MYO	Dice RV	LVEF error (points)
Siemens (A)	yes	20	0.936	0.856	0.893	3.6
Philips (B)	yes	50	0.920	0.874	0.894	4.4
GE (C)	no	50	0.890	0.826	0.858	5.3
Canon (D)	no	50	0.904	0.824	0.875	3.8
All		170	0.908	0.843	0.878	4.4 (median 3.0)

- Reliability policy (decided Nov 7): scanner shift alone is a caveat on the review, not an escalation; a severe shift (beyond 99% of clean validation exams from all four vendors, score 14.3) is always escalated. Thresholds come from clean data only.
- Policies compared on 170 clean + 85 fault-injected exams:

Policy	Faults caught	Clean exams flagged	Escalations appropriate	Wrong predictions missed
No agent (baseline)	0%	0%	–	60
Shift always escalates	100%	55%	57%	5
Shift = caveat	77%	21%	75%	12
Shift = caveat, severe shift escalates (chosen)	92%	25%	75%	9

- Chosen policy by fault: noise and slice drop 100%, contrast 90%, blur 76%.
- Fixed on the way: 3 held-out subjects had skewed NIfTI orientation that nnU-Net could not read; converter now cleans it.
- Outputs: 170 clean + 255 fault-injected exam JSONs in the shared format, with uncertainty maps.

ECG (Ana Karen)

- [x] ~~PTB-XL: one binary task (e.g. normal vs abnormal) on lead I, patient-wise official folds~~
- [x] ~~Small 1D CNN with MC-dropout or a 5-model ensemble for uncertainty~~
- [x] ~~Signal quality index (e.g. NeuroKit2) and heart-rate plausibility~~
- [x] ~~Map PhysioNet 2017 labels to the same task, resample, and measure the performance drop (device shift)~~

- [x] ~~Embedding-distance shift score; export cached outputs~~

ECG results (Nov 7, run on SDRE H200, code in `ecg/`):

- PTB-XL lead I, 21,791 ECGs, folds 1–8 train, 9 calibrate, 10 test; 5-model 1D-ResNet ensemble.
- Test AUROC 0.887 (95% CI 0.874–0.901). PhysioNet 2017 AliveCor: AUROC 0.673 overall, 0.776 for normal vs AF only.
- Reliability v2 (template match, baseline, QRS-band and high-frequency power; calibrated entropy uncertainty; embedding + recording-feature shift score; all thresholds from clean validation ECGs): injected faults 100% caught (v1 72%), 81% of clean ECGs accepted (v1 86%).
- Accuracy on accepted ECGs 0.82 vs 0.79 on all; the most certain half reaches 0.95.
- Device shift score separates AliveCor from PTB-XL with AUROC 0.94; 90% of noisy AliveCor recordings flagged.
- Limits: PTB-XL expert-annotated real noise on lead I is flagged only 23% vs 16% for unannotated ECGs; on AliveCor, uncertainty does not separate right from wrong (AUROC 0.50).
- Cached outputs: one shared-format JSON per ECG (clean, faulty, PhysioNet 2017). Agent's ecg_max_uncertainty (0.35) needs the calibrated value 0.98.

Agent and dashboard (Vertika)

- [x] ~~Policy engine: thresholds in a YAML config, deterministic accept / repeat / escalate~~
- [x] ~~LLM tool-calling orchestrator (quality, uncertainty, shift, plausibility, draft-review tools), first on mock outputs~~
- [x] ~~Case tracker in SQLite with the four states, simulated clock, stalled-case reminders, audit log of every action~~
- [x] ~~Dashboard views: clinician queue, case timeline, equity audit with an "Approve safeguard" button~~
- [x] ~~Template fallback for review drafts if the LLM call fails~~

Agent results (Nov 7; simulated Medly-style cohort of 2,001 patients x 90 days, real ECG and MRI pipeline outputs, same assumptions for both arms):

Measure	HeartSafe	Medly alone
Median hours, first alert to acknowledgement (true episodes)	7.8	13.9

Measure	HeartSafe	Medly alone
True episodes acknowledged within 24 h	77%	69%
Alerts caused by faulty readings stopped	98%	0%
Nurse queue items per 100 patient-months	112	123
False alarms per 100 patient-months	86	99
Escalations resolved within deadline	97%	87%
Median hours to resolution	26.5	28.1
Stalled cases found	693 of 696	0 of 736

- Real model evidence inside the agent: faulty ECGs kept out of the evidence 100% (84% of clean ECGs used); unreliable MRI results flagged 67% (83% of good ones accepted). The agent now uses each pipeline's calibrated decision instead of its own thresholds.
- LLM orchestrator: Claude calls read-only tools (case facts, data quality, ECG, MRI, follow-up) and writes the review via submit_review; decisions stay deterministic; each run is in the audit log; template fallback when there is no key. Tested with a stand-in client only, not yet against the live API.
- Dashboard: patient journey, clinician queue, equity audit with approve-and-apply safeguards, results vs Medly alone, model reliability (ECG and imaging results, policy comparison).
- Demo patient follows the demo script: scale change gets a repeat, missed days get outreach, clean abnormal ECG escalates, uncertain regional-scanner MRI goes to review, stalled case gets reminders.
- Assumptions to state on the slide: which patients get faulty ECGs or unfamiliar scanners, clinician response times and forgetting rates (agent/config.yaml).

Equity and evaluation (Yassmin + Ana Karen)

- [x] ~~Subgroup metrics with bootstrap 95% CIs for both models~~
- [x] ~~Gap rule: flag a subgroup when its CI sits below the overall estimate; propose the safeguard~~
- [x] ~~Simulated cohort and baseline runs (see Evaluation)~~

Equity results (Nov 7), bootstrap 95% CIs, gap = subgroup CI entirely worse than overall; every safeguard waits for administrator approval:

- Imaging (LV Dice, LVEF error by vendor, centre, sex, age band): 3 gaps, all on LV Dice: GE 0.890 (never seen in training), centre 4 (the GE centre) and age 60+ 0.888, vs 0.908 overall. No LVEF-error gap.
- ECG (accuracy, unreliable-data rate by sex, age band, device, site): 6 gaps, including lower accuracy for age 40–59 and device CS-12 E, and higher unreliable-data rates for age 60+, device CS-12 and site 2. No sex gap in either model.
- Agent equity monitor: ECG unreliable-data rate 0.43 in remote communities vs 0.25 urban (gap flagged). Approving the tighter-deadline safeguard for remote patients cuts their mean time to acknowledgement from 11.4 h to 7.7 h, at the cost of 489 extra reminders and 0.3 extra false alarms per 100 patient-months.

Evaluation

We compare the agent with a no-agent baseline (every prediction accepted, no follow-up tracking) on one simulated cohort built from the held-out test sets. To get ground truth for "unreliable", we inject known faults into a share of cases: added noise, flat or clipped ECG segments, cross-device ECGs, and out-of-vendor or degraded MRI slices.

Metric (from abstract)	How we measure it	Owner
Detection of unreliable predictions	Sensitivity / specificity of repeat-or-escalate on injected faults	Ana Karen
Appropriate escalation	Share of escalations where the model was wrong or the input was faulty	Ana Karen
Subgroup gap detection	Gaps flagged vs gaps present in subgroup CIs	Yassmin
Unresolved cases found	Stalled cases flagged vs cases simulated to stall	Vertika
Follow-up completion	% of escalations resolved within the simulated window	Vertika
Time to resolution	Median simulated days, agent vs baseline	Vertika
Extra false alarms	Added escalations after the safeguard is approved	Yassmin

Clinician response delays are simulated from a simple assumed distribution; we state that assumption on the slide.

Demo and presentation

The talk follows one patient through Medly plus HeartSafe, then zooms out to the equity audit and the numbers. HeartSafe is presented as the layer that makes Medly-style monitoring safer, not a replacement for it.

1. Problem: heart failure care in remote communities; Medly works, but alerts depend on reliable data and timely follow-up.
2. What Medly already does (credit it), and the gap: unreliable inputs, alerts without a structured review, lost follow-ups, no subgroup monitoring.
3. Live demo, one patient in a remote community:
    a. Medly flags weight gain; HeartSafe sees a scale change and missing days, and asks for a repeat reading instead of escalating.
    b. The repeat confirms the gain and a good-quality ECG is abnormal; escalation with an explainable review.
    c. MRI at the regional centre has high uncertainty; clinician review requested.
    d. The case stalls, a reminder fires, the clinician resolves it, and it is documented.
4. Equity audit: remote communities have a higher unreliable-data rate; a safeguard is proposed and an administrator approves it.
5. Results against Medly alone.
6. Responsible AI: the clinician decides, HeartSafe complements follow-up visits, and data use follows OCAP® in partnership with communities.

Record a 2–3 minute backup video of the demo on Nov 11 in case the live run fails.

Risks and cut list

The biggest risks are data access and nnU-Net training time, so both start on Nov 6.

Risk	Fallback
M&Ms download needs registration and may be delayed	Request access today; fall back to a smaller public cardiac MRI set with vendor info

Risk	Fallback
nnU-Net training too slow	2d config, one fold, fewer epochs; demo runs on cached predictions
Integration slips past Nov 10	Demo on cached model outputs; the agent logic stays live
LLM API fails or costs too much	Template-based review drafts, same flow
Subgroup CIs too wide to show a gap	Report it honestly; demonstrate the safeguard loop on the largest observed gap
Nov 11 is Remembrance Day	Confirm everyone's availability at kickoff

If we are behind, cut in this order: real notifications → live LLM root-cause text → live imaging inference → ECG Grad-CAM. Never cut reliability checks, escalation, follow-up tracking or the equity loop; they are the pitch.

Mentor touchpoints and submission checklist

Book three short sessions with the mentor now, so feedback lands before each decision point.

- **Nov 6 – scope check (20 min):** Is the MVP the right size? What do judges weigh most? Are the escalation rules clinically sensible?
- **Nov 10 – demo review (30 min):** Walk through the end-to-end flow at feature freeze.
- **Nov 12 – full rehearsal (45 min):** Timed run of slides and demo.

Before submitting on Nov 13

- [ ] Confirm the exact deadline time, time zone and submission format
- [ ] Code repo with README, setup steps and the config file of thresholds
- [ ] Slides (PDF copy too)
- [ ] Backup demo video
- [ ] Evaluation table and subgroup results with CIs
- [ ] Responsible-AI note: de-identified public data only, clinician makes every decision, audit log
- [ ] Dataset and method citations (PTB-XL, PhysioNet 2017, M&Ms, nnU-Net)
- [ ] Team #40 names and emails on the title slide